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Weekend Healthcare Pulse: Surf's up or is that a Blue Wave - Our Healthcare Policy Outlook as we enter midterm season

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At this distance, based on the latest polls, it looks likely that Democrats will take control of the House, while the Senate remains a toss-up. For purposes of this note, we are focused on Democrats taking control of at least the House in the mid-terms. We will be holding an event on Sept 1st discussing what a blue Wave could mean for healthcare policy post-midterm and in 2028 (Click here for registration).

What is the policy outlook if Democrats start taking back control? When looking at how Democrats may shift the policy environment, we see four major policy topics 1) Reversing Trump era policies; 2) Expanding coverage; 3) Improving affordability; and 4) Potential emerging areas of focus in AI and vertical integration. We do not expect any major policy changes before the mid-terms. Post mid-terms, if Democrats take control of one or both chambers of Congress, the major policy focus will likely be reversing (or delaying) Trump era health care cuts in Medicaid and Marketplace. We see investors beginning to look for the post-2028 environment during 2027 as the primaries warm up on both sides of the aisle. We believe the major topics to be discussed for the 2028 campaign will include expanding coverage (on the Democratic side) and improving affordability (likely for both parties).

Our top five predictions for policy priorities if Democrats perform strongly in the mid-terms. 1) We see revival of enhanced subsidies for Marketplace as one of the more likely policy changes after the mid-terms. 2) We see potential for additional delay or reversal of a subset of Medicaid cuts in the 27/28 time period as well. 3) We believe the 2028 Democratic primaries will include a debate over how best to expand coverage, with public option our expected victor. 4) We see new affordability policy proposals as being important priority and likely will involve limiting/capping out-of-pocket expenses for consumers. 5) We see policies to break up vertical integration as becoming more important post-2028.

Implications - We see a Democratic win of the House (and potentially Senate) as benefiting Medicaid MCOs and to a lesser extent hospitals. We do not expect incremental policy risk to PBMs or employer MCOs, but could see MA as seeing limited risks of policy changes such as elimination of Stars quality bonuses. For names, we see MOH, CNC, ELV and HCA as benefiting from expectations of a Democratic victory in November. We see MA as having some valuation headwinds from a Democratic win, and this might impact HUM more than other names due to concentration and expectation that recovery of Stars bonuses will be bigger source of growth.

We are announcing a Policy Symposium virtual event on Sept 1st 10am ET - 12 noon focused on what a Democratic sweep means for policy (Click here for registration), both post-mid terms and into the primaries. We will be hosting experts from across the spectrum of Democratic healthcare policy to discuss policy objectives and what can be implemented.

By: Lance Wilkes, Kevin Peterson, Adam Chow

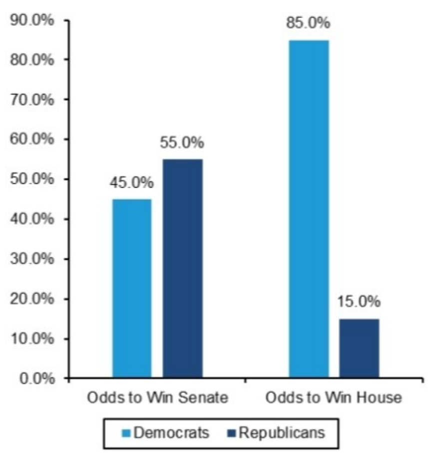
www.bernsteinresearch.com
First Published: 20 Jul 2026 04:01 UTC Completion Date: 17 Jul 2026 19:59 UTC

At this distance, based on the latest polls, it looks likely that Democrats will take control of the House, while the Senate remains a toss-up. For purposes of this note, we are focused on Democrats taking control of at least the House in the mid-terms, and then a shift in focus to Democrats aiming to win the Presidency, and potentially the Senate, in 2028. We will hold an event on Sept 1st to discuss what a blue Wave could mean for healthcare policy post-midterm and in 2028 (Click here for registration).

Republicans are expected to lose control of the House, as Republican currently have a small 5 seat edge (based on '24 vote) and the history of seat losses during midterms for the President's party. Redistricting complicates the equation, but most prediction markets expect Democrats to take control of the House. The margin of potential victory is less important in the House, but prediction markets expect around 15-20 seats. While this would seem a smaller pickup than other time periods given the President's low approval ratings, we see around 200 seats as roughly a floor for Republicans, based on polls, and therefore would not expect a much larger pickup by Democrats.

For the Senate, ME and NC are the states expected to most likely swing Dem, given ME voting for Harris in 24 and NC retirement of Tillis and a strong candidate in former Governor Cooper. The next two states are less likely, with OH, AK and TX the more probable toss-ups. Movement of these states would signify a strong blue wave, which certainly is possible, but will be difficult to call all the way up to the election given that these states went for Trump by over 10 points. Prediction markets, such as Polymarket, suggest Republicans retain control but the odds are very even.

EXHIBIT 1: **Polymarket odds to win Senate and House**



Source: Polymarket, Bernstein analysis

EXHIBIT 2: **Senate - key races for 2026 midterms**

State	Party in Control of reelection seat	Incumbent running for Senate?	Margin of Victory for President			Margin for Senate Elections
Republican seats most at risk:			**2024**	**2020**	**2016**	**2020**
ME	Republican	Yes - Susan Collins	D +6.9%	D +9%	D +3%	R +8.6%
NC	Republican	No - Thom Tillis	R +3.2%	R +1.34%	R +3.7%	R +1.8%
OH	Republican	Yes - Jon Husted	R +11.2%	R +8.0%	R +8.5%	R +6.1%
IA	Republican	No - Joni Ernst	R +13.2%	R +8.2%	R +9.4%	R +6.5%
TX	Republican	No - John Cornyn	R +13.7%	R +5.6%	R +9.0%	R +9.6%
Democratic seats to watch:						
MI	Democrats	No - Gary Peters	R +1.4%	D +2.8%	R +0.2%	D +1.7%
GA	Democrats	Yes - Jon Ossoff	R +2.2%	D +0.2%	R +5.1%	D +1.2%
NH	Democrats	No - Jeanne Shaheen	D +2.8%	D +7.4%	D +0.4%	D +15.6%

Source: Ballotpedia, Bernstein analysis

What is the policy outlook if Democrats start taking back control? When looking at how Democrats may shift the policy environment, we see four major policy topics 1) Reversing Trump era policies; 2) Expanding coverage; 3) Improving affordability; and 4) Potential emerging areas of focus in AI and vertical integration.

Reversing Trump policies - Following midterms, we see three priorities for Democrats: 1) Reversing Trump policies 2) Improving affordability 3) Expanding healthcare coverage. We believe initial focus will be on reversing Trump policies such as renewing the enhanced Marketplace subsidies and overturning aspects of the OBBBA. Democrats will seek to renew the prior enhanced subsidy. They may seek to obtain this by offering a compromise to restore the higher subsidy amount but also to increase the availability of HRAs in order to win Trump's signature.

Democrats will also seek to loosen Medicaid work requirements. We are already seeing a push against this, as last month 25 states plus D.C. sued the Trump administration over CMS' implementation of Medicaid work requirements. Democrats could seek to delay the implementation of work requirements as the recent lawsuit argues the interim rule departs from initial guidance, after states had already invested significant resources updating eligibility systems, staffing, and beneficiary communications based on prior CMS guidance.

Other OBBBA Medicaid funding cuts could be targeted as well, including provider taxes. Similar to work requirements, Democrats will seek to limit or delay the reduction of provider taxes. States with large Medicaid programs and significant reliance on provider taxes would likely be a particular focus of legislative relief efforts.

Democrats could seek to make cuts to MA in order to fund their priorities. Potential MA targets include risk adjustment, home risk assessments, and Star quality bonuses. A recently introduced Democrat-sponsored bill targets MA funding cuts in all of these areas. Additional scrutiny could focus on chart reviews, coding intensity, and favorable selection. While direct MA cuts would be politically challenging, reforms to payment methodology could be used as a way to generate savings that can be redirected towards Marketplace and Medicaid expansion or strengthening traditional Medicare benefits.

Background on Trump policy -The key policies in OBBBA are1) work requirements (community engagement) which mandate work-like effort and documentation, and are expected to increase uninsured by 5+m; 2) provider tax reform and state directed payments, which limits provider tax safe harbors and is expected to impact 1+m enrollees, and hospital finances; and 3) enhanced eligibility redetermination for Marketplace. In addition, the Trump Administration has increased overall verification and fraud/abuse efforts which have led to reductions of Medicaid enrollment of around 1% per quarter since the 2nd quarter of 2025.

EXHIBIT 3: **OBBBA Impacts on Medicaid spending and enrollments - CBO estimates**

OBBBA Provision affecting Medicaid	Implementation period	Spending cuts 2025-2034	Impact on Medicaid enrollment	Uninsured population increase in 2034
Work requirements	Jan 27 - Jan 29	$317.0 bn	5.7 Mn	5.3 mn
Provider taxes	2028 - 2032	$182.7 bn	2.4mn (1.3mn ACA)	1.1 mn
State directed payments	Jan 2028	$149.4 bn	na	na
ACA eligibility redeterminations	Jan 2027	$58.0 bn	10% of coverage	700k
Moratorium on CMS rules	-	MSP - $66.0 bn CHIP - $53.6 bn	MSP - 800k for Medicaid duals	CHIP - 400k
Other	-	$60.1 bn	na	na
Total	-	**~$886 bn**	-	**7.5 mn**

Source: CBO, Bernstein analysis

Expanding coverage policies - Expanding coverage will be another priority for Democrats.

One option for Democrats is to restore funding to Medicaid and Marketplace in an effort to achieve peak enrollment achieved through COVID and enhanced Marketplace coverage. ACA membership peaked at 24.3m in 2025, and estimates predict it could fall to 16.5-17.5m this year. After reversing Trump policies, Democrats may seek to expand eligibility to an even greater level and eventually surpass the 24m peak in Marketplace membership.

A second option to expand coverage is through federal or state public options. A federal public option has seen advocacy before, including the proposed Affordable CHOICE Act earlier this year. 3 states already have public options, with CT recently announcing a proposal for their own state option. We see this type of public option as basically leveraging the Marketplace chassis, with the following additions: 1) shifting provider reimbursement to government reimbursement or a specified premium to Medicare/ Medicaid; 2) expanding subsidies either as part of enhanced subsidies or to further augment coverage; 3) expanding access to small employer and possibly state/municipal workers. We could also see further expansion to allow employees to switch over to public option and have portability for the coverage. We note that Republicans could propose something like this through an expansion of ICHRA with some tax credits or subsidies for types of workers.

Connecticut Governor Ned Lamont had proposed a “Connecticut Option” - a public health plan designed by the state and run by private insurers. Gov. Lamont proposed the plan as a part of his reelection campaign policy and introduced legislation which directs Office of Policy and Management to test the feasibility of the proposal.

Lamont specifically stated that Connecticut will not be a traditional “public option” where the state takes on risk and also administers the plan. The overall structure of the Connecticut option is based on the program run by Colorado state from 2023. The Colorado option program has provided lower premiums and 15% lower out-of-pocket costs on average compared to non-Colorado option plans. Similar to Colorado’s state option plan, and the other state options in WA and NV, the plan does not involve the government bearing underwriting risk. The plan would be designed by the state, but run by private insurers.

The third option we see is Medicare for All, another method of coverage expansion, but we believe this is a less likely scenario, as Democratic support has seemingly shifted from Medicare for All to expansion of current programs like ACA. However, support for Medicare for All does still exist, and a candidate supporting that platform is likely to make a presidential run in 2028, which could lead to MCO volatility. Currently, there are 17 Senators (all with Democratic caucus) who are co-sponsors of the Medicare for All bill. The incoming class could impact this, but it seems candidates have mixed objectives, including former Senator Brown from OH who is a challenger this election, switching from Medicare for All to focusing on public option.

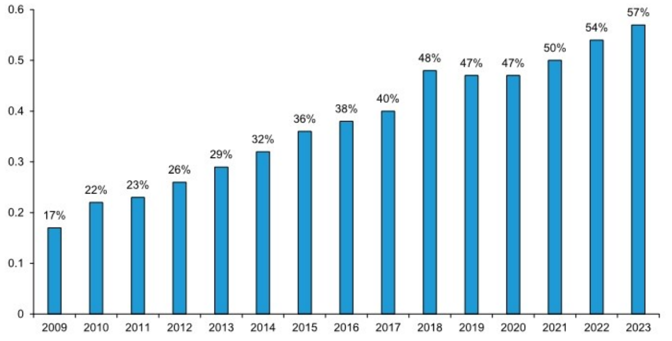
EXHIBIT 4: **Senate candidates’ position on Medicare for All**

State	Retiring Senator	Position on M4A	Dem Poll Leader	Position on M4A
MI	**Gary Peters (D)**	**Did not co-sponsor M4A bills**	**El-Sayed (D)**	**Supports M4A**
			Stevens (D)	Does not support M4A
NC	Thom Tillis (R)	Opposes M4A	Cooper (D)	Does not support M4A
OH	Jon Husted (R)	Opposes M4A	Brown (D)	Does not support M4A
ME	**Susan Collins (R)**	**Opposes M4A**	**TBD**	**TBD**
MN	**Tina Smith (D)**	**Sponsored and supports M4A.**	**Flanagan (D)**	**Supports M4A**
IA	Joni Ernst (R)	Opposes M4A	Josh Turek (D)	Does not support M4A
GA	Jon Ossoff (D)	Did not co-sponsor M4A bills	Jon Ossoff (D)	Did not co-sponsor M4A bills

Source: Candidate web pages, Bernstein analysis

Improving affordability - While many will portray any policy as improving affordability, we see this topic as high on voter priorities. We therefore expect further policy development to achieve affordability. High deductible plans and other shifts in policy structure have contributed to this affordability issue to consumers.

EXHIBIT 5: **% of covered individuals enrolled in a plan with a deductible of $1000 or more**



Source: KFF Health System Tracker, Bernstein analysis

For Democrats this could be accomplished through legislation to cap out-of-pocket medical expenses, limit deductibles, or provide additional subsides. Over the past 20 years a number of policies have been adopted including out of pocket maximums for employee and ACA policies, insulin caps and the Inflation Reduction Act's limit on Part D out-of-pocket cost for Medicare patients. Democrats have also previously focused on more comprehensive insurance which limits consumer cost exposure, but increases the cost of the policy. We would expect further efforts on this side to limit out of pocket costs and risks. We could see a specified amount of out of pocket which is paid monthly, rather than when a service is incurred.

What is most likely from a timing perspective? We do not expect any major policy changes before the mid-terms. Post mid-term, if Democrats take control of one or both chambers of Congress, the major policy focus will likely be reversing (or delaying) Trump era health care cuts in Medicaid and Marketplace. We see investors beginning to look for the post-2028 environment during 2027 as the primaries warm up on both sides of the aisle. We believe the major topics to be discussed for the 2028 campaign will include expanding coverage (on the Democratic side) and improving affordability (likely for both parties).

Potential MA reform proposals - We see MA as a potential source of savings to fund healthcare expansion in a Democratic set of proposals. While risk adjustment may be a source of savings, we see this as largely addressed by existing policies and policy direction. Another feature mentioned in the Save Medicare Act of 2026 bill proposal is ending Stars quality bonus payment. In general, we see MA savings as a lower likelihood priority for Democrats but believe other forms such as limiting prior authorizations might be part of a healthcare package.

Our top five predictions for policy priorities if Democrats perform strongly in the mid-terms. 1) We see revival of enhanced subsidies for Marketplace as one of the more likely policy changes after the mid-terms. 2) We see potential for additional delay or reversal of a subset of Medicaid cuts in the 27/28 time period as well. 3) We believe the 2028 Democratic primaries will include a debate over how best to expand coverage, with public option our expected victor. 4) We see new affordability policy proposals as being an important priority and likely will involve limiting/capping out-of-pocket expenses for consumers. 5) We see policies to break up vertical integration as becoming more important post-2028, as both progressives and populists may seek to break apart the large healthcare players.

Implications - We see a Democratic win of the House (and potentially Senate) as benefiting Medicaid MCOs and to a lesser extent hospitals. We do not expect incremental policy risk to PBMs or employer MCOs, but could see MA as seeing limited risks of policy changes such as elimination of Stars quality bonuses. For names, we see MOH, CNC, ELV and HCA as benefiting from expectations of a Democratic victory in November. We see MA as having some valuation headwinds from a Democratic win, and this might impact HUM more than other names due to concentration and expectation that recovery of Stars bonuses will be bigger source of growth.

UPCOMING 2026 TEACH-INS AND EVENTS

We are announcing a Policy Symposium virtual event on Sept 1st 10am ET - 12 noon focused on what a Democratic sweep means for policy (Click here for registration), both post-mid terms and into the primaries. We will be hosting experts from across the spectrum of Democratic healthcare policy to discuss policy objectives and what can be implemented.

We are hosting a teach-in 7/30 at 11am ET on Medicaid Managed Care (Click here to register). This session will cover the history, present, and future outlook of managed Medicaid including potential policy implications.

Our recent Specialty Pharmacy teach-in provided a detailed overview of the specialty pharmacy industry, including its history, economics, competitive dynamics, policy aspects, and outlook. Here is the REPLAY and SLIDES.

Medicaid Managed Care 7/30 at 11am ET Click here to register

PBM & Employer self-insured 8/20 at 11am ET Click here to register

OptumInsights 8/27 at 11am ET Click here to register

BERNSTEIN TICKER TABLE

			16 Jul 2026		TTM		Adjusted EPS			Adjusted P/E (x)		
			Closing	Price	Rel.							
Ticker	Rating	Cur	Price	Target	Perf.	Cur	2025A	2026E	2027E	2025A	2026E	2027E
AGL (agilon health)	M	USD	121.22	86.00	124.8%	USD	(24.46)	(4.81)	(4.52)	(5.0)	(25.2)	(26.8)
CNC (Centene)	O	USD	63.89	68.00	107.2%	USD	2.08	3.38	4.23	30.6	18.9	15.1
CI (Cigna)	O	USD	283.82	381.00	(24.5)%	USD	29.84	30.49	33.75	9.5	9.3	8.4
CVS (CVS)	O	USD	106.50	106.00	50.3%	USD	6.75	7.37	8.14	15.8	14.5	13.1
ELV (Elevance)	O	USD	372.85	488.00	3.3%	USD	30.30	27.09	30.60	12.3	13.8	12.2
HCA (HCA)	M	USD	385.74	413.00	(15.5)%	USD	28.21	30.44	33.14	13.7	12.7	11.6
HUM (Humana)	O	USD	386.48	425.00	60.1%	USD	16.60	9.12	15.41	23.3	42.4	25.1
MOH (Molina)	O	USD	224.82	286.00	(9.7)%	USD	11.03	5.24	11.80	20.4	42.9	19.1
UNH (UNH)	O	USD	423.38	492.00	32.4%	USD	16.35	18.86	21.99	25.9	22.4	19.2
SPX			7,457.69									

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

Source: Bloomberg, Bernstein estimates and analysis.

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Market-Perform (Bernstein Brand) Neutral (Autonomous Brand)	HOLD	35.8%	16.2%
Underperform	SELL	13.1%	13.6%

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